Health Affairs Scholar, 2025, **3**(5), qxaf091
https://doi.org/10.1093/haschl/qxaf091
Advance access publication: April 28, 2025
Research Article

Health Affairs Scholar
EMERGING & GLOBAL HEALTH POLICY
OXFORD

Medicaid expansion increased income among newly eligible adults

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Abstract

The Affordable Care Act's Medicaid expansion improved health care access for low-income Americans; however, its impact on economic outcomes—particularly income—is less clear. We used US Census administrative income data that tracked 6120 cohorts covering 84% of working-age adults from 2005 to 2019. Using difference-in-differences, we compared changes in income among low-income adults living in expansion states, before and after expansion, with changes in income in low-income adults living in non-expansion states. Low-income adults living in Medicaid-expansion states experienced an average 9.5% relative increase in income in the 5 years after expansion. This impact was concentrated among adults who were likely newly eligible for Medicaid after the expansion. These adults experienced a 9.6% relative increase in income, as well as a 2.1 percentage point (7.3%) relative higher likelihood of having income in the 40th income percentile or higher. We found suggestive evidence that a reduction in unpaid time off from work drove a small part of this relative income increase among those newly eligible. While Medicaid funding faces substantial political uncertainty, this evidence suggests that welfare gains to newly eligible individuals include economic, as well as health, improvements.

Key words: Medicaid; Medicaid expansion; income; economic outcomes.

Introduction

The expansion of Medicaid eligibility to low-income US adults through the Affordable Care Act (ACA) has resulted in substantial gains in health insurance coverage for a population at high risk of being uninsured.[1] This has led to improvements in health care access, quality of care, and health outcomes among low-income populations living in states that elected to expand Medicaid.[2-9]

Medicaid expansion's impact on economic outcomes is less clear, both conceptually and in practice. On the one hand, increased access to Medicaid may increase labor market participation and subsequent income through better management of medical conditions or increased income limits allowing enrollees to earn more due to reduced income constraints.[10,11] On the other hand, providing access to health insurance coverage that is decoupled from employment may discourage labor market participation, and thus reduce income. Existing research reflects this ambiguity. While some work found little to no effect of Medicaid enrollment on short-term labor force participation and wages,[12-17] other work found evidence of changes in employment and employment match quality.[18-23]

To advance our understanding of the relationship between Medicaid access and economic outcomes, we used a novel dataset generated from linked demographic and tax records of the majority of working-age adults in the United States from 2005 to 2019 to quantify the effect of Medicaid expansion on income. This dataset, which aggregated records to create unique cohorts of shared demographic, geographic, and economic characteristics, has several advantages over survey data used in prior work, such as its use of administrative records (instead of respondent recollection) to report income, as well as its national sample (as opposed to nationally representative subsample).

We further examined the impact of Medicaid expansion separately on individuals who were likely to have been eligible for Medicaid prior to expansion and those who were likely to be newly eligible after the expansion to explore whether expansion impacted these populations differently. Finally, we used publicly available data from a national survey containing information on labor market participation to explore potential drivers of any changes in income.

Data and methods

Data

Our primary dataset was the Mobility, Opportunity, and Volatility Statistics (MOVS) data, a longitudinal (2005–2019), administrative dataset created from linked demographic and tax records of over 95 million working-age adults in the United States who were 25–50 years old in 2005.[24] These data are aggregated into 6120 cohorts of individuals by sex, race-ethnicity, state of residence in 2005, and base (2005) income decile (additional details provided in Appendix B.1).

The MOVS data include several cohort-year measures of income collected from the Internal Revenue Service's tax records and the Department of Housing and Urban Development. Sources of income included pretax "wages, self-employment income, ordinary dividends, social security benefits, and rental income."[24] There are 3 key features of this dataset that made

Received: February 26, 2025; Revised: April 23, 2025; Accepted: April 25, 2025
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it particularly suitable for our study. First, it contains administrative records of the near-complete population of Americans, which helps mitigate reporting bias commonly found in income survey data.[25] Second, cohorts can be tracked over time, facilitating our empirical strategy (two-way fixed effects). Third, due to the data's longitudinal nature, a cohort's Medicaid eligibility can be identified prior to Medicaid expansion, thus avoiding endogeneity concerns around Medicaid eligibility and income manipulation.

Measures

Medicaid expansion states were classified based on 2014 expansion status. The 23 expansion states included Arizona, Arkansas, Colorado, Delaware, Hawaii, Illinois, Iowa, Kentucky, Maryland, Massachusetts, Michigan, Nevada, New Hampshire, New Jersey, New Mexico, New York, North Dakota, Ohio, Oregon, Rhode Island, Vermont, Washington, and West Virginia. To avoid contamination of our estimates by earlier- and later-expanding states, we dropped the 18 states that expanded before (California, Connecticut, Washington, DC, and Minnesota) or after (Alaska, Idaho, Indiana, Louisiana, Maine, Missouri, Montana, Nebraska, North Carolina, Oklahoma, Pennsylvania, South Dakota, Utah, and Virginia) 2014 in our main analyses. We included a subset of these states in a sensitivity analysis that accommodated staggered treatment timing. The 10 non-expansion states included Alabama, Florida, Georgia, Kansas, Mississippi, South Carolina, Tennessee, Texas, Wisconsin, and Wyoming. Appendix Table A1 provides additional details on our characterization of states by expansion status.

We defined each cohort's Medicaid expansion status based on their 2005 state of residence, the only residency state available (we discuss limitations of this approach below). We characterized cohort Medicaid eligibility based on the ratio of 2009 equivalized income to the 2009 Federal Poverty Level (FPL). Equivalized income represents the portion of household income available to each working-age household member, and can be used to capture individual income (adjusted for household composition). Cohorts living in expansion states were Medicaid eligible if their ratio was below the state-specific Medicaid eligibility threshold. Cohorts living in non-expansion states with incomes up to 138% FPL (the threshold set by the ACA) were characterized as eligible. We used 2009 income to mitigate any bias from individuals manipulating their income following the passage of the ACA in 2010 in order to be Medicaid eligible. We checked the robustness of our results to using 2009 income by defining Medicaid eligibility based on 2013 income. Additional details on this approach are given in Appendix B.1.

The ACA's Medicaid expansion significantly increased income eligibility thresholds for parents, from 106% to 138% FPL on average and, for the first time, allowed low-income childless adults to enroll.[26] To better understand whether Medicaid expansion may have differentially impacted different populations, we disaggregated the Medicaid eligible population into (1) those who were previously eligible for Medicaid (if their income-to-FPL ratio was at most the state-specific, pre-expansion threshold for working parents in 2013) and (2) those who became newly eligible for Medicaid after the expansion (if their income-to-FPL ratio fell between the 2013 working parent threshold and the post-expansion threshold). We used working parent thresholds since the average individual in our sample had a household with at least 1 child. We restricted our analytical sample to cohorts whose mean income assigned them to 1 of these 2 categories, and excluded cohorts with a mean income above the expanded Medicaid eligibility threshold.

To explore potential drivers of changes in income, we used the Survey of Income and Program Participation (SIPP). SIPP is a nationally representative longitudinal survey that contains a stratified sample of 53 070 housing units. We used the 2014 SIPP panel, which included survey responses from 2013–2016 and allowed us to track individuals across time and place. SIPP contains monthly information on labor market participation and participation in government-assistance programs.

Our main outcome of interest was individual income; as is convention in literature evaluating policy impacts on income, we used the log-transformed version of this measure. Evaluating changes in income allowed us to explore the impact of Medicaid expansion on economic indicators that represent growth relative to the individual themselves (ie, how does my income post-expansion compare to my income pre-expansion).

A secondary outcome of interest was income mobility; changes in income percentile provide an economic indicator of growth relative to others (ie, how does my income post-expansion compare to others' income post-expansion), or a measure of an individual's economic mobility. Specifically, we captured economic mobility via the proportion of individuals in a given cohort-year who were in the 40th income percentile or higher. Percentiles were assigned based on income-by-birth-year cohort, allowing for age-adjusted comparisons.

A third class of outcomes were potential drivers of income, observed in the SIPP data. First, we captured labor market participation through examining the probability that a respondent was employed (in general or part-time) in any given month. Additionally, we examined the probability that a respondent had unpaid time away from work in a given month, which may reflect time off due to illness. Alternatively, a change in unpaid time away from work may signal a change in the quality of the job held by the individual. Second, we captured government sources of income through receipt of supplemental security income (SSI) and other disability pay, which may reflect whether the respondent had a disability that kept them from working.

Study design

Using an event study design, we compared within-cohort changes from before (2009–2012) to after (2014–2019) expansion in outcomes of interest among low-income working-age adults in states that expanded Medicaid ("treated group"), relative to outcome changes among low-income working-age adults in states that did not expand Medicaid ("control group"). This approach estimated dynamic treatment effects for each year, relative to a reference year (2013).

Models included standard two-way fixed effects for calendar year and cohorts, which allowed us to account for any secular trends in income as well as any unobservable, time-invariant characteristics of the cohort, respectively. We also included controls for time-varying changes in household structure—the number of children, number of adults, and marital status—that may have been related to income. We clustered standard errors at the state level since it is the unit of treatment assignment. All analyses included population weights assigned to each cohort.

A central assumption in such empirical models is that, but for the treatment (ie, Medicaid expansion), outcomes among the treated group would have evolved similarly to outcomes among the control group. While this assumption is inherently

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untestable, our event study allowed us to check for parallel trends between the 2 groups in the pre-expansion period, providing support to the causal nature of our estimates. In robustness checks, we also restricted our estimates to states that did not increase their minimum wage in 2014, to avoid conflating the impact of a simultaneous minimum wage increase with the impact of expanded Medicaid eligibility on income.

To gain precision in these estimates, and to obtain 1 summary measure of the effect of Medicaid expansion, we estimated a difference-in-differences version of our main model, which included 1 pooled pre-period (years 2009–2013) and 1 pooled post-period (years 2015–2019), omitting 2014 as a "transitional" year. Additional details on our empirical approach are provided in Appendix C.

Sub-analyses

We conducted several additional sub-analyses. To explore heterogeneity, we separately estimated our model by cohort sex and race/ethnicity. To assess whether the impact of Medicaid expansion in 2014 was generalizable to other years, we estimated a pooled difference-in-differences model (allowing for staggered event timing) that included all states that had expanded Medicaid between 2010 and 2018,[27] and separately estimated difference-in-differences models for early (2010 and 2011) and late (2015 to 2018) expanders. We also relaxed our strict "never treated" control requirement and included states that expanded Medicaid after 2019 as control states.

Limitations

This study had several limitations. First, state of residence was only identified in 2005. Thus, individuals assigned to the treated group may no longer have been living in a Medicaid expansion state at the time of expansion, and analogously for those assigned to the control group. If movement between states was unrelated to income, then this measurement error would have attenuated our estimates, leading us to underestimate the effects of expansion on income.

If movement between states was correlated with both income and the direction of the move, then this relationship may partially explain the (relative) change in income. To address these concerns, we estimated our event study on the proportion of a cohort (1) that moved between states each year and (2) that no longer live in their 2005 state (Appendix Figure A1). There were no significant differences in year-to-year movement between states (panel a). However, there was a significant increase in the likelihood of treated groups living in states different from their 2005 residence following Medicaid expansion, although we were unable to determine the directionality of the move (ie, from non-expansion to expansion state). Regardless, including this measure as a control in our main specification did not result in meaningfully different estimates. Furthermore, prior work has shown little to no evidence of a relationship between the establishment of social programs (such as Medicaid expansion) and interstate migration.[28] Finally, we used the Panel Study of Income Dynamics database to examine geographic mobility over time among individuals residing in expansion states in 2005, and found that, even as late as 2019, approximately 88% of adults remained in an expansion state (Appendix Figure A2).

Second, we were not able to observe the number of individuals in a given cohort who actually enrolled in or were (previously or newly) eligible for Medicaid. However, our focus on cohorts of working-age adults whose average income in 2009 was at most the state-specific Medicaid-qualifying income threshold increased the likelihood that individuals in our sample were among the population(s) of interest.

Relatedly, individuals in cohorts whose income was below the eligibility threshold in 2009 may no longer have been eligible around the time of expansion, weakening any observed ties between Medicaid expansion and income. The direction of the bias arising from this weakened relationship is unclear. If increases in income among non–Medicaid-eligible individuals were due to other contemporaneous shocks outside of Medicaid expansion, then our estimates would be biased upward. If instead only Medicaid expansion increased income, our estimates would be biased downward.

Appendix Figure A3 plots the share of cohorts in expansion and non-expansion states that were Medicaid-eligible in 2014 (based on mean 2009 income) and that remained eligible, by year, based on their income-to-FPL ratio in that year. Shares are weighted by cohort population. By 2014, 85% of cohorts in expansion states and 92% in non-expansion states were still eligible, and by 2019, 70% and 81% were still eligible, respectively. We also checked the robustness of identifying eligibility based on 2009 income by re-estimating our model on treated and control groups whose Medicaid eligibility was determined by their 2013 income.

Third, we were only able to observe income reported to the Internal Revenue Service. To the extent that individuals in our sample did not report their full income, our estimated effects may underestimate the true effect of Medicaid expansion on income. Furthermore, should individuals intentionally underreport their income to remain eligible for Medicaid, our estimated effects may be biased towards the null.

Finally, we were unable to observe the income sources in our primary data. This limited our ability to attribute changes in income to a specific pathway, such as labor market participation. We used SIPP data to explore labor market participation and receipt of income from other government programs.

Results

In general, treated and control groups were very similar in demographic and household characteristics over our sample period (Table 1). The average age was 38 years, and slightly over half were women. Hispanic (any race) and White (non-Hispanic) individuals comprised nearly 80% of the sample, with more White (non-Hispanic) individuals in expansion states. The composition of the average household was approximately 4 people (2 adults and 2 children), and approximately 40% of the sample was married. Income measures were not meaningfully different between the 2 groups. Appendix Table A2 provides additional details about the previously and newly eligible subpopulations.

Unadjusted trends in income illustrate that, prior to 2014, income was largely in parallel between treated and control groups (Figure 1). After 2014, the increase in income among treated individuals appeared slightly greater relative to control individuals. Without accounting for potential confounding factors between treated and control groups, there was a relatively clear deviation in trends that began around 2015. We next rigorously tested whether this deviation was significant using our event study approach, as well as whether the difference in pre-trends was nonsignificant.

Figure 2A plots the coefficients estimated from our event study, with 95% CIs. There were no obvious pre-trends,

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Table 1. Characteristics of the sample, 2009.

	Expansion	Non-expansion
Mean (SD) age in base year, y	38 (1)	38 (1)
Female, %	54	56
Race/ethnicity, %		
Hispanic, any race	19	52
White non-Hispanic	68	32
Black non-Hispanic	11	15
AIAN non-Hispanic	0	0
Asian non-Hispanic	1	0
Other/multiple race non-Hispanic	0	0
Household composition, mean (SD)		
No. of persons	4 (0.53)	4 (0.49)
No. of adults	2 (0.21)	2 (0.18)
No. of children	2 (0.38)	2 (0.38)
Married, %	40	39
Change in state of residence from 2005, %	13	12
Income, mean (SD)		
Family income, $	38 498 (11 328)	37 249 (9712)
Individual income, $	19 641 (5337)	19 050 (4802)
Income-to-FPL ratio	0.98 (0.24)	0.96 (0.23)
Bottom income quintile, %	46 (18)	46 (18)
Top income quintile, %	3 (2)	2 (1)
No. of people (1000s)	10 815	9346
No. of cohorts	942	405
No. of states	23	10

Source: Authors' analysis of data from Mobility, Opportunity, and Volatility Statistics (MOVS) from the US Census Bureau. Notes: All values except for the bottom 3 rows of totals are weighted by the number of individuals in a given cohort.
Abbreviations: AIAN, American Indian and Alaska Native; FPL, Federal Poverty Level.

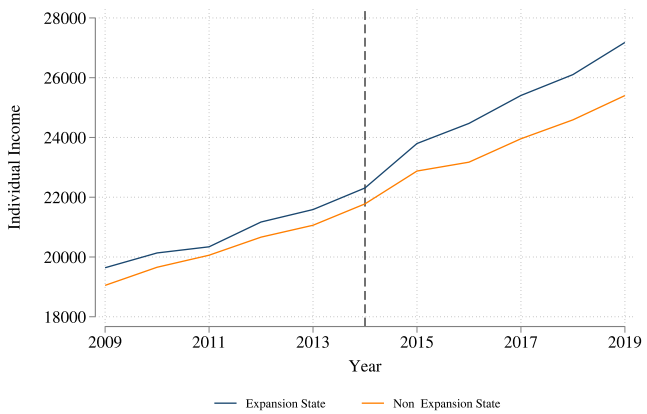

Figure 1. Mean individual income by expansion status, 2009–2019. Source: Authors' average yearly estimates from the Mobility, Opportunity, and Volatility Statistics (MOVS) data, Notes: 2009–2019. Means are unadjusted and weighted by the count of individuals in a given cohort. The vertical dashed black line represents the year of Medicaid expansion (2014).

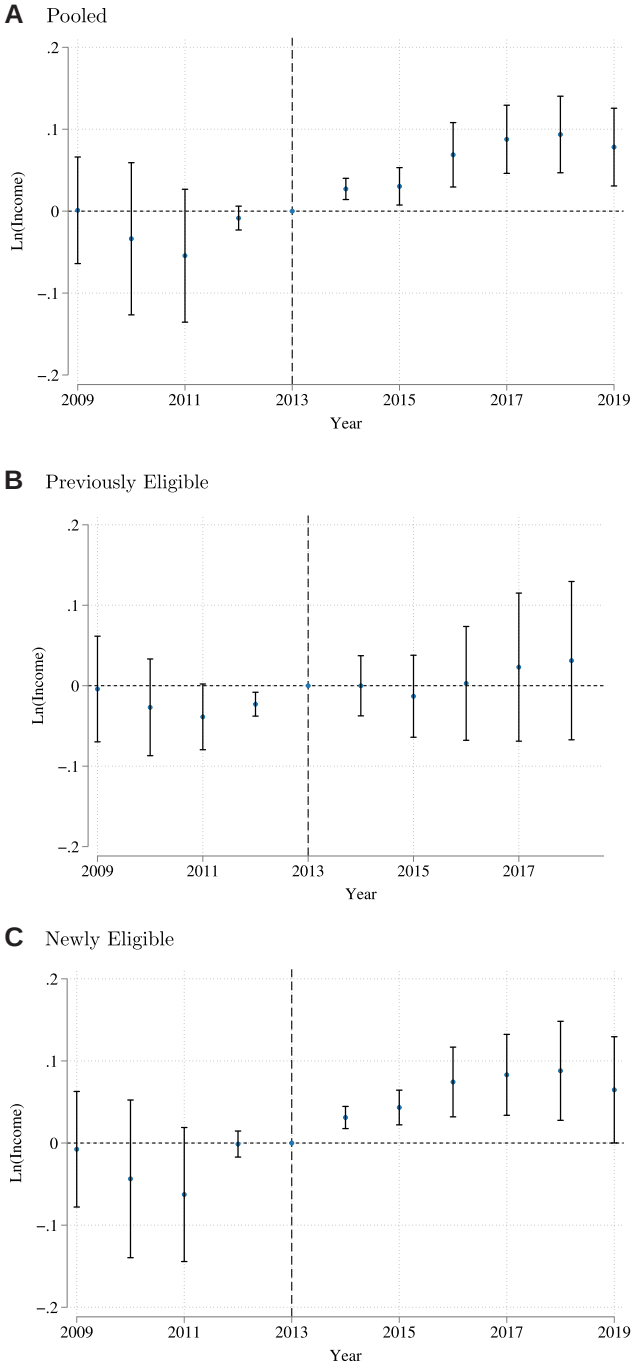

Figure 2. (A–C) Relative change in log individual income among low-income individuals in expansion vs non-expansion states. Source: Authors' average yearly estimates from the Mobility, Opportunity, and Volatility Statistics (MOVS) data, 2009–2019. Notes: 95% CIs are reported around each point estimate. The vertical dashed black line represents the reference year, i.e. the year before Medicaid expansion (2013).

supporting our identification strategy. In the year immediately following expansion, treated individuals experienced an additional 2.8% (95% CI: 1.5%–4.0%) increase in income relative to control individuals. This relative increase rose through the end of the study period, at which point the relative change in income among treated individuals was approximately 8.1% (95% CI: 3.3%–13.2%) greater than in control individuals.

Figure 2B and C show the differential impact of Medicaid expansion on income among previously eligible and newly eligible individuals, respectively. While there was no significant effect of Medicaid expansion on income among previously eligible individuals, there was a clearer relative increase among newly eligible individuals that generally increased over time.

Panel A of Table 2 summarizes these effects using our difference-in-differences approach. Medicaid expansion

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Table 2. Difference-in-differences estimates.

	(1) Pooled	(2) Previously eligible	(3) Newly eligible
Panel A. Log individual income			
Treated × Post-expansion	0.090*** (0.020)	0.038 (0.050)	0.092*** (0.026)
Observations	188 792 645	67 363 288	121 429 357
Pre-expansion control mean	$20 088	$15 925	$20 620
Pre-expansion treated mean	$20 566	$20 100	$21 176
Panel B. Share with income above the 40th percentile			
Treated × Post-expansion	0.016*** (0.003)	0.005 (0.004)	0.021*** (0.006)
Observations	188 792 645	67 363 288	21 429 357
Pre-expansion control mean	0.235	0.149	0.246
Pre-expansion treated mean	0.249	0.242	0.259
Panel C. Probability of being away from work without pay			
Treated × Post-expansion	−0.003 (0.013)	0.025 (0.018)	−0.039** (0.017)
Observations	47 232	25 128	22 104
Pre-expansion control mean	0.047	0.06	0.039
Pre-expansion treated mean	0.06	0.053	0.071

Source: Panels A and B estimates obtained from the authors' analysis of Mobility, Opportunity, and Volatility Statistics (MOVS) from the US Census Bureau data. Notes: Panel C estimates obtained from the authors' analysis of the Survey of Income and Program Participation (SIPP) data. Standard errors are reported in parentheses and are clustered at the state level. All models include cohort and year fixed effects, as well as controls for the proportion of the cohort that is married, mean number of adults in the household, and mean number of children in the household. Models using MOVS data are weighted by cohort size; models using SIPP data are unweighted. Year 2014 is omitted from the "post" period as a transition year. In Panel A, pre-expansion control and treated means are reported using unadjusted individual income.
*$P < .1$.
**$P < .05$.
***$P < .01$.

increased income among treated individuals by an additional 9.5% (95% CI: 5.2%–14.0%), on average, in the post-expansion period, relative to the control (column 1). While there was a small 3.9% (95% CI: −5.7% to 14.5%) relative increase in income for previously eligible treated individuals, this result was not significant (column 2). Finally, relative to newly eligible control individuals, Medicaid expansion increased income among newly eligible treated individuals by 9.6% (95% CI: 4.1%–15.4%).

Panel B of Table 2 reports the impact of Medicaid expansion on economic mobility. The share of individuals whose income rank rose above the 40th percentile increased by 1.6 (95% CI: 0.9–2.3) percentage points (pp) in the pooled sample, although there was no impact on previously eligible populations. Newly eligible individuals experienced a significant 2.1-pp (95% CI: 1.0–3.2-pp) increase in income rank (8.1% relative to the mean). The corresponding event study shows no pre-trends (Appendix Figure A4C).

Sub-analyses

While the impact of Medicaid expansion on income was similar for female and male cohorts, the impact on Black cohorts (18.0%; 95% CI: 10.5%–26.0%) was substantially larger than the impact on other races and ethnicities (Appendix Figure A5). The overall impact of Medicaid expansion including non-2014 expansions was an approximately 6.5% (95% CI: 4.4%–8.6%) relative average increase in income (Appendix Table A3). This effect was largest among early expanders, and nonsignificant overall in later expanders. Notably, there was a significant decrease in income among previously eligible cohorts in late-expanding states, although this was driven by 1 state (Alaska) in which there were significant downward pre-trends. Thus, we do not consider this to be a causal result.

Sensitivity analyses

To test whether the relative increase in income among treated individuals could have been driven by factors other than Medicaid expansion, we conducted 2 additional tests. First, we dropped 3 states that increased minimum wage by at least $0.25 from 2013 to 2014.[29] Appendix Figure A6 shows that the model estimates were qualitatively unchanged from the main results.

Second, we conducted a falsification test in which we estimated our model on cohorts whose mean income was increasingly greater than the state's Medicaid threshold. Appendix Figure A7 demonstrates that, in cohorts with average income at least 150% of that state's Medicaid threshold, the effects of expansion on income are null. This suggests that we are likely capturing the effect of Medicaid expansion on individuals who are income eligible, even if there is a range of incomes within each cohort.

Additionally, we tested the sensitivity of our results to the year of income used to define Medicaid eligibility, and redefined eligibility using 2013 (instead of 2009) income. Appendix Figure A8 shows results similar to the main estimation. Finally, due to the small number of clusters in our model, we conducted a wild cluster bootstrap exercise and confirmed that the event study coefficients in the post-period remained significant (Appendix Table A4).

Potential mechanisms

To explore potential drivers of relative increased income among newly eligible treated individuals, we estimated our model on an analogous population of Medicaid-income–eligible individuals in SIPP data. We found no evidence of a change in labor market participation in terms of any (Appendix Figure A9) or part-time (Appendix Figure A10) employment. However, we found a substantial reduction in the probability that a newly eligible individual had to take time away from work without pay after Medicaid expansion (Appendix Figure A11); this effect grew over time, from a reduction of approximately 42.1% (2014) to a reduction of nearly 68.3% (2016), although notably, only the 2016 impact was significant at the 10% level. While the annualized results for newly eligible respondents were supported by monthly-level event studies, the significant increase observed for previously eligible cohorts in 2016 was not.

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Finally, we found no clear evidence of any change in income derived from sources such as supplemental security income (Appendix Figure A12) or disability pay (Appendix Figure A13).

Discussion

Relative to low-income individuals living in states that did not expand Medicaid but who would have been newly eligible for Medicaid under expansion, this study found that Medicaid expansion increased the income of newly Medicaid–eligible individuals by an additional 9.6% in the 5 years following the implementation of the ACA. These findings are aligned with prior work attributing Medicaid to keeping millions of individuals out of poverty.[30]

Existing studies have found mixed results of Medicaid expansion on employment.[13,20-23] We found little to no impact on workforce participation but a reduction in unpaid time off from work, providing a different lens on the economic impacts of expansion. While there are multiple explanations for the reduction in unpaid time off (such as switching to a higher quality job), 1 common reason for taking unpaid time off is care-taking for others or personal illness/medical issues.[31] Gaining Medicaid coverage personally or within the household may allow individuals to better manage health care conditions that had previously kept them out of work. However, using a back-of-the-envelope calculation (Appendix D), we found that this reduction likely explained at most 12% of the increase in income, leaving drivers of the vast majority of this increase still unexplained. We leave this for future work.

Our findings of both income increases and upward mobility among low-income adults in Medicaid-expanding states add to the relatively large body of evidence demonstrating that access to Medicaid improves welfare along a variety of different dimensions. This new evidence is particularly relevant given current political efforts to drastically reduce federal spending on Medicaid.[32] For example, during the first few months of the Trump administration, Congress has begun to consider a reduction in the amount of federal funding that states receive for covering the health care costs of Medicaid enrollees who became eligible due to the expansion.[33] In the context of our findings, reducing federal support for the continued Medicaid enrollment of these populations may introduce unintended economic consequences, impacting financial stability and income for this population.

Furthermore, our results may also provide relevant insights for the recently resurrected discussions around conditioning Medicaid enrollment on employment.[34] Despite recent reports that 92% of working-age adult Medicaid enrollees are working full or part-time (or are not working due to factors such as caregiving responsibilities, illness or disability, or school attendance),[35] lawmakers are currently considering Medicaid work requirements as another potential mechanism to cut federal Medicaid spending. Combined with evidence showing that Medicaid work requirements have little to no impact on increases in employment, and are likely to generate substantial coverage losses (largely due to administrative reporting burden),[35-37] our findings provide new insight that policies geared towards increasing workforce participation among Medicaid enrollees may not only be unnecessary but may also be harmful.

Conclusion

Individuals who became eligible for Medicaid after their state expanded income thresholds in 2014 experienced a significant increase in multiple economic indicators relative to other low-income individuals in non-expansion states. We found suggestive evidence that this increase was driven, in part, by a reduction in unpaid leave from work. When weighing the costs of providing health care coverage to additional populations against potential individual and social benefits, policymakers and other stakeholders should consider this new evidence of significant economic gains through expanding access to Medicaid.

Acknowledgments

The authors gratefully acknowledge William Dow, Tamara Hayford, Madeline Helfer, Kendall Houghton, Gina Li, Matthew Staiger, and Jessica Van Parys for their helpful comments and suggestions.

Supplementary material

Supplementary material is available at *Health Affairs Scholar* online.

Funding

This work was unfunded.

Conflicts of interest

We have no conflicts of interest.

Data availability

The datasets underlying this article were derived from publicly available sources described in Appendix B. The data underlying this article will be shared on reasonable request to the corresponding author.

Notes

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